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"Robbed of my life": The Felt Loss of Familiar and Engaged Presence in Depersonalization/Derealization Disorder

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Abstract

Depersonalization/derealization disorder (DPDR) is classified as a dissociative disorder in the DSM5. It is noteworthy that the symptoms of depersonalization and derealization are commonly found in many other psychological disorders, including schizophrenia spectrum disorders, while phenomenological features of schizophrenia are commonly found in DPDR. The current study attempts to clarify these apparent similarities via highly detailed phenomenological interviews with four persons diagnosed with DPDR. The data revealed four interrelated facets: 1, Loss of resonance, 2, Detachment from experience, 3, Loss of self, and 4, Commitment to reality. These facets point to a *felt loss of immediate and familiar engagement in experience* as a basic organizing Gestalt which permeates the various symptoms and signs of our participants with DPDR. Close consideration of this disruption allows their experiences to be more easily distinguished from those of schizophrenia-spectrum disorders.

Keywords

depersonalization/derealization disorder – dissociative disorders – schizophrenia – phenomenology – phenomenological psychopathology – qualitative research

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1 Introduction

Depersonalization/derealization[1] disorder (DPDR) is a psychiatric disorder involving profound and debilitating experiences of detachment from one's immediate experiences of oneself and the surrounding world. Despite significant suffering and disability arising from this condition, and despite its long-standing recognition as a form of psychopathology, it remains understudied and under-diagnosed, with relatively slow progress in developing effective forms of assessment and treatment. Although there are numerous and detailed accounts of pathological symptoms and altered subjective experiences, there are few models that describe how and why these diverse experiences relate to one another, and what distinguishes their presence in DPDR from their common occurrence in other disorders. Phenomenology offers one approach that can meaningfully address these questions, and thereby contribute to more sensitive and specific assessment tools, suggest etiological factors, and identify potential targets for treatment. The present study applies a phenomenological approach to a small group of DPDR patients, each of whom was interviewed at considerable length and in a highly detailed, in-depth fashion.

DPDR is classified among the dissociative disorders in the DSM-5, and reflects pathological "experiences of unreality, detachment, or being an outside observer with respect to one's thoughts, feelings, sensations, body, or actions" or "with respect to surroundings" (American Psychiatric Association, 2013, p. 302). It can also include a number of related features, such as "mind-emptiness" (feeling like one has no thoughts), heightened self-observation, and altered time experience. Depersonalization and derealization are also common symptoms that can be found in a number of other disorders, including depression, panic disorder, post-traumatic stress disorder, and schizophrenia spectrum disorders (APA, 2013; Sierra, 2009, Simeon, Knutelska, Nelson, & Guralnik, 2003), and DPDR may frequently be comorbid with other syndromes, such as depressive and anxiety disorders (Sierra, 2009). DPDR is thought to have a lifetime prevalence of about 1–2 percent (APA, 2013; Hunter, Sierra, & David, 2004), with onset typically in the late teens or early 20s (Baker et al., 2003, Simeon et al., 2003), and to affect an approximately equal proportion of men and women (Simeon et al., 2003). Despite its relative prevalence (approximately equivalent to schizophrenia and bipolar disorder), the disorder continues to be under-diagnosed, and it can take many years to get a correct diagnosis following the onset of symptoms (Michal et al., 2016).

One of the more intriguing features of DPDR is the maintenance of reality testing in the face of strong feelings of unreality of oneself and the surrounding world; by definition, it excludes any form of psychosis. This is interesting

because depersonalization and derealization can also play a major role in psychotic symptoms and disorders. For example, 48% of patients with schizophrenia spectrum disorders demonstrated pathological levels of depersonalization (Haugen & Castillo, 1999); inpatients with psychosis scored higher on a measure of depersonalization than controls or relatives (Gonzalez-Torres et al., 2010); and depersonalization and/or derealization are correlated with severity of hallucinations and delusions (Perona-Garcelán et al., 2010), paranoia (Černis et al., 2014), and first-rank symptoms (Humpston, Harrow, & Rosen, 2020). These and similar findings have led to the development of several different models of the relationship between depersonalization (and derealization) and psychosis or schizophrenia spectrum disorders, including:

1. DPDR is actually a schizophrenia spectrum disorder (Parnas & Handest, 2003);
2. Psychotic symptoms are fundamentally dissociative phenomena (Longden, Madill, & Waterman, 2012);
3. Depersonalization increases vulnerability to psychosis (though this may be distinct from "primary" psychotic disorders; Allen, Coyne, & Console, 1997);
4. Depersonalization is simply a common psychiatric symptom, found in numerous disorders outside the dissociative disorders, and is but one of many features involved in the development of psychosis (Sierra, 2009);
5. Depersonalization is essentially related to the structure of schizophrenia, though it can be found in other disorders and be a disorder in its own right (Sass et al., 2018).

Additional research on dissociative disorders more generally finds at least superficial similarities between psychotic and dissociative disorders (Ciaunica, Pienkos, Nakul, Madeira, & Farmer, 2022), potentially leading to misdiagnosis and incorrect treatment, though other psychological features, treatment response, and the specific nature of symptoms are useful in accurate diagnosis.

One key question that remains is whether the qualitative features of DPDR are distinct from the forms of depersonalization and derealization found in psychotic disorders, and whether the experiences themselves can be practically distinguished. One study (Sass, Pienkos, Nelson, & Medford, 2013) provided some initial clarification of this issue by reviewing published first-person accounts and clinical descriptions of DPDR to determine whether characteristic phenomenological features of schizophrenia, as identified by the Examination of Anomalous Self Experience (EASE; Parnas et al., 2005), were described by persons with DPDR. The EASE comprises various manifestations of disordered selfhood, often described as disturbances of ipseity or basic or minimal self, and reflects the predominant phenomenological theory

of schizophrenia. Total scores on the interview have reliably demonstrated differences between persons with and at risk of developing schizophrenia and those with non-schizophrenia spectrum disorders (including bipolar disorder, non-schizophrenia psychotic disorders, mixed non-psychotic diagnoses, and non-clinical samples; Haug et al., 2012; Nordgaard & Parnas, 2014; Raballo, Saebye, & Parnas, 2011). Sass et al. (2013) found that a vast majority of EASE items could be identified in descriptions of DPDR, although certain items relating to a more profound confusion of self-other boundaries and self identity were not present. It should be noted that the methodology used precluded attempts to provide total scores on the EASE, so the total *number* of self disorders in DPDR could not be compared to schizophrenia spectrum disorders. However, the apparent presence of many potential forms of self disturbance in DPDR calls into question the specificity of this hypothesis – at least as typically formulated – to the schizophrenia spectrum.

The current study attempts to address this issue by conducting highly detailed, semi-structured phenomenological interviews with several persons diagnosed with DPDR to identify the qualitative features of their experiences and how they relate to an experiential core or characteristic Gestalt of the disorder.[2] The goals of this paper are twofold: 1, to add to the available literature on the phenomenology of DPDR; 2, to clarify, if possible, the phenomenological distinctions between DPDR and schizophrenia spectrum disorders. These in-depth interviews and subsequent analysis have the potential to provide much-needed clarity to subtle but important qualities of subjective experience in DPDR.

2 Materials and Methods

2.1 *Participants*

This study recruited participants who met the following criteria: diagnosis of depersonalization/derealization disorder (or equivalent, e.g., depersonalization disorder) by a psychiatrist or clinical psychologist, between ages 18–64, and interested and able to describe a variety of experiences associated with DPDR as demonstrated in response to screening questions. Exclusion criteria included a primary diagnosis of substance/medication-induced psychopathology, or symptoms due to another medical condition, as well as inability to provide information on past subjective experiences. No potential participants met these exclusion criteria and so none were excluded from this study.

Participants were recruited from two sources: 1) past interactions with study authors, during which they expressed interest and ability to speak about past

experiences of psychopathology; or 2) online message boards that focus on discussions of experiences of psychiatric diagnoses. Emails and flyers soliciting participation were sent following IRB approval, and interested participants were invited to speak with the first author to determine eligibility for the study and proceed with the interview if these criteria were met (see below).

Four participants were included in this study; an additional participant dropped out of the study following data collection. All four participants identified as male, and the mean age at time of interview was 33.3 years old (ranging from mid-twenties to early fifties). Participants had experienced the symptoms that had led to their diagnosis of DPDR for between 15 months and more than 30 years.

2.2 *Data Collection*

All participants provided written informed consent to participate in the study. Participants were then screened for eligibility. All participants who contacted the first author were found to meet inclusion criteria and were then asked basic questions about their psychiatric history, including history of being assigned a diagnosis of DPDR, history of any previous diagnoses, and brief history of psychiatric symptoms. This was followed by the EASE and EAWE interviews (see below). Participants met twice with the interviewer to complete both interviews. Interviews were conducted either in person (one participant) or over Skype or other video-enabled Voice-Over Internet Protocol (VOIP; three participants). All interviews were audiotaped and/or videotaped and transcribed. Following analysis of the interviews, participants were given the option to provide feedback on their interview transcripts and analysis, which was incorporated into the final results. All participants were given the option to take breaks or stop the interview at any time, and information regarding mental health resources was available if participants expressed significant distress or desire for mental health support (though no participants requested this).

2.3 *Measures*

Interviews were conducted using two phenomenologically-oriented interviews, the Examination of Anomalous Self Experience (EASE; Parnas et al., 2005) and the Examination of Anomalous World Experience (EAWE; Sass et al., 2017). Both of these interviews are semi-structured, providing a list of items/questions to use in the interview, while allowing for flexibility to ask follow-up questions to get additional detail. In general, semi-structured phenomenological interviews are recommended for obtaining more valid information about the experience of psychiatric illness, especially schizophrenia (Nordgaard, Sass, & Parnas, 2013).

The EASE interview is commonly used to assess self-disturbances in schizophrenia and other disorders and has shown strong reliability. Studies so far conducted have shown high specificity and sensitivity to schizophrenia spectrum disorders and persons at high risk for developing schizophrenia, at least in comparison with bipolar disorder, non-schizophrenia psychotic disorders, mixed non-psychotic diagnoses, and non-clinical samples (Parnas et al., 2005; Schultze-Lutter et al., 2007; Haug et al., 2012; Nordgaard & Parnas, 2014; Raballo, Saebye, & Parnas, 2011).

The EAWE is a relatively new interview that was developed for the purpose of investigating changes in the experience of the world in schizophrenia, including experiences of space and objects, time and events, other persons, language, and overall "atmosphere." The EAWE has demonstrated strong inter-rater reliability (Conerty et al., 2017) and can reliably distinguish between patients with non-affective first-episode psychosis and non-patients (Madeira et al., 2019). It has also been used effectively in qualitative research to provide detailed information about experiences of the world in schizophrenia and other disorders (Englebert et al., 2019; Pienkos, Silverstein, & Sass, 2017). In this study, one participant was interviewed using an early (pre-publication) version of the EAWE, and three participants were interviewed using the published version.

The interviewer and first author is a co-author of the EAWE and was involved in data collection and analysis of EAWE reliability data, and she has received basic and advanced training in the EASE. The second author is a co-author of the EAWE and has received basic training in the EASE. Both authors have extensive training and research expertise in phenomenological psychopathology and its clinical applications. The interviews took place over two sessions (one for the EASE and one for the EAWE), with each session lasting approximately 1 ½ hours (ranging from 47 minutes to 2 hours 19 minutes).

2.4 *Data Analysis*

Data was analyzed using methods for descriptive phenomenological analysis, described in depth by Giorgi (2009) and further clarified in other texts, including Wertz (2005, 2011) and Giorgi (1975, 1997). Descriptions of the method emphasize several basic attitudinal requirements for engaging in data analysis, particularly 1) engaging in the "psychological phenomenological reduction," which shifts the researcher's attention to the nature of the psychological processes under investigation (rather than attending to external objects or events); 2) "bracketing the natural attitude," that is, suspending commonsense assumptions and beliefs about the object or phenomena being studied (also called the *epoché*), to focus on the nature of the phenomena as they occur to the participant; and 3) developing a general or invariant structure of the

psychological phenomena, which is investigated via the "eidetic reduction": imaginatively varying the constituents or features of a phenomenon to determine the essential, defining features of the phenomenon as it was lived.

The basic steps of the descriptive phenomenological method involve 1) immersion in the data by reading a transcript several times to get an overall sense of meaning; 2) identifying statements relevant to the phenomena in question and separating these statements into "meaning units" that each reflect a distinct psychological phenomenon (though some phenomena will be discussed in multiple meaning units) – that is, where there is a shift from describing one phenomenon (e.g. feelings of isolation) to another (e.g. feelings of low self-worth); 3) transforming the commonsense or natural-attitude descriptions in each of the meaning units into universal or essential expressions of the phenomenon of interest, through the use of free imaginative variation; and 4) synthesizing these phenomenological expressions into a meaningful description of the structure of the phenomenon.

When there are multiple participants, this procedure is completed with an initial participant and then repeated with the others. These individual analyses are then compared to identify common themes that reflect various aspects of what is taken to be a common psychological process, which are used to develop one or more general or typical structural descriptions common to all (or to a subset of) participants (Giorgi, 1997). This is the point at which the experimenters' interpretations play the largest role, given that deciding on the most accurate generalization necessarily involves judgmental decisions that are hardly automatic. For this study, this involved reviewing each of the facets of each individual structure to identify points of commonality, and then arranging these shared facets in logical relationship with one another. For example, features from individual structures termed "decreased vividness," "everything flat, dull," and "changing sensitivity" were grouped together in a facet called "dulled perception," which was then linked with other facets such as "loss of mattering," "strangeness of the world," etc. in the larger theme of "Loss of resonance," described below. This was followed by comparing the general structure to each of the individual structures and transcripts to assess whether the experiences of individual participants were consistent with the general structure. As noted above, participants provided feedback on both the individual and general structures and whether these captured their experiences in a meaningful way.

Explicit focus on the specific items of the EASE and EAWE interview formats, as such, did not play the primary role in this analysis. That is, the analysis focused on the participants' descriptions offered in the interviews, and on how these accounts could be understood to reflect various experiences of self and

TABLE 1 EASE and EAWE scores

	Total possible score	Mean (sd)	Range
Total EASE score	57	16.25 (2.22)	13–18
Domain 1: Cognition and stream of consciousness	17	4.50 (1.91)	2–6
Domain 2: Self-awareness and presence	18	8.00 (1.41)	7–10
Domain 3: Bodily experiences	9	2.50 (1.91)	1–5
Domain 4: Demarcation/transitivism	5	0.75 (1.50)	0–3
Domain 5: Existential reorientation	8	0.50 (0.58)	0–1
Total EAWE score*	75	17.25 (7.5)	10–27
Domain 1: Objects and space*	17	3 (2.45)	0–6
Domain 2: Time and events*	6	3.25 (1.71)	1–5
Domain 3: Other persons*	14	2.75 (1.5)	1–4
Domain 4: Language*	10	2.5 (1.91)	0–4
Domain 5: Atmosphere*	17	4.75 (0.96)	4–6
Domain 6: Existential orientation*	11	1 (1.15)	0–2

* Includes scores from one participant assessed with an early version of the EAWE; this version does not include some items included in the published version of the EAWE.

world and their meaningful relations to one another, *rather than* on the a priori organization (the items or domains) of the interviews. A similar approach has been used in previous qualitative publications using the EASE and the EAWE, revealing insights into the structure of world experience in schizophrenia (Pienkos et al., 2017) and important phenomenological differences between individuals with schizophrenia (Englebert et al., 2019).

Quantitative scores on the interview were also calculated, with a score of 1 given for items that were assessed as "definitely present," and 0 given for items that were assessed as "not present" or "possibly present." Total scores, standard deviations, and ranges for the EASE and EAWE and each of their domains are presented in Table 1.

3 Results

The following results describe a common underlying structure that was found across the EASE and EAWE interviews with all four participants, though important individual differences are mentioned when relevant. This basic structure

can be formulated as a *felt loss of immediate and familiar engagement in experience*. The four interrelated facets of this structure that emerged (as "common themes" – see just above) are: 1, Loss of resonance, 2, Detachment from experience, 3, Loss of self, and 4, Commitment to reality. In this section we offer summaries of each of these facets, along with illustrative quotations from participants. We then will consider their relationships to the overarching Gestalt in the Discussion.

3.1 *Loss of Resonance*

The first facet or common theme is a lack of emotional response or connection to events or objects in the world. The world feels fake or deadened, objects do not have immediate value or relevance, and people, places, and objects evoke no feelings of familiarity or any emotion at all. Nothing is physically changed, and participants recognize what things are, but the personal *feel* of everything is gone. One participant described this as, "objects look kind of like dead. Everything looks quite kind of lifeless. Nothing evokes any emotions in me. It's kind of like everything I look at just gives me the same experiences of blackness."

The world can feel colorless, evanescent, cold, or overly bright and sterile: such are the features of a world that does not resonate; participants may feel as if there is no self with whom it *can* resonate. Similarly, other human beings appear to lose their immediate sense of humanity, and instead seem like objects or machines, sometimes experienced as uncanny, or grotesque in their materiality or organicity. A participant described experiencing his mother as

> like a robot, a robot that looked like my mother, and I was fascinated with the way her mouth was moving, and the way the sound was that came out. So even though my consciousness was taking in on some level what she was talking about, I was mostly the other part of me, just looking on in fascination at this strange opening, it's like a strange opening that has these two shapes that we call lips, but I didn't think about them as being lips, and when she smiled and laughed it was kind of repulsive, that this aperture is making that.

Other people can also seem as though they are merely acting: their actions and words appear meaningless and fake, rather than expressive of an inherent intentionality.

Because participants feel they are not a part of the world, there is a loss or lack of reality, which participants could describe as akin to living in a dream, being trapped in a bubble, or lost in space. The term "reality" can be ambiguous, however. One participant stated that because of the loss of subjective

coloring, things can seem *more* realistic than before, given that his perceptions and interpretations were not being distorted by past experiences, expectations, or emotional states.

In addition, objects and the general atmosphere can be experienced as much less vivid or intense during DPDR. This can be described as though there were a screen or a film between the participant and the rest of the world so that the world feels less crisp and more distant. This was noticed by at least one participant with respect to every sensory modality. All participants experience this as a change in themselves and their perception, rather than a change in the features of the world itself: the world feels less interesting or appealing, or they no longer experience it as colored with their affective responses to it.

Associated with these shifts in perception is the sense of the world as no longer appealing or mattering to oneself. This can manifest in several ways. There may be a change in one's ability to concentrate and focus on things: individuals may never feel fully engaged or connected to things, so nothing stands out or captures the attention. Everything is equally distant, vague, unremarkable, or useless. As a result, one might be much more easily distracted by extraneous or irrelevant stimuli, and it can feel fatiguing to focus for long periods.

One participant described an even more profound experience of detachment, such that he no longer notices the commonsense meaning or relevance of objects, but instead becomes fascinated by their physical properties, such as formal visual details or the peculiar intonations of someone speaking:

> So I look around, and I say, hmm, that's a chair, what a strange shape it is, you know, and I even look at my beloved little dog, and I think, what an odd thing … How weird that vowel sound is. How strange that sounds to me, and that sound is coming from that person.

On the other hand, one participant finds it easier to focus because of being less distracted by apparently irrelevant or intrusive emotional reactions.

There can also be a loss of motivation or drive because of this diminished sense of "mattering": Since there is no self for whom worldly objectives are relevant or important, it becomes difficult or impossible to feel the concern or internal drive to accomplish things in the world. As one participant noted, "[I] completely lack motivation and desire to do things. I also feel very fatigued a lot, tired … kind of feel like a dead feeling, like never want to do anything … everything is forced, everything requires effort …" Three of the four participants noted that this loss of mattering seems to decrease their concern about the outcomes of their actions, reducing anxiety or stress in situations that would, in the past, have been anxiety-provoking or stressful. Interestingly, however,

there can also be increased anxiety, related in these cases to being unable to cope with situational demands or respond adequately in social situations, or to the nature and trajectory of the DPDR experiences.

Furthermore, participants described a loss of the ability to feel emotionally attached to others, or in some cases, of the desire to have a strongly felt emotional attachment. They can (and do) still maintain relationships with close friends, family, and romantic partners, but experience a feeling of distance and lack of immediate emotional response in these and other relationships. For example, "It's just that there's no emotions. I don't get any emotions from anybody. Or like when I see people speak, it's just it seems a bit fake, a bit strange, because I don't feel anything when I see them." Rather than having an intuitive response to others, participants described a sense of having to consciously act out appropriate behaviors in social situations, something that many find exhausting or stressful. One participant also noted that the experience of kindness or warmth from others functions to diminish his experience of DPDR, as it can prompt him to experience strong attachment-related emotions.

3.2 *Detachment from Experience*

The second facet or common theme described by participants with DPDR concerns feelings of detachment from all subjective experiences, typically involving a loss or diminishment of the sense that my own experiences are truly or fully happening *to me*. This was often described either as sensing a kind of barrier between oneself and one's experiences, or as having somehow lost a self that could truly experience things.

Participants described thoughts as not coming from them, and often as having fewer or slower thoughts, having difficulty accessing their thoughts, or not having any thoughts at all. They can also feel disconnected from their ongoing speech, feeling that their words did not seem to be connected to any thoughts they could easily identify. This can result in needing to exert great effort in order to know how to respond. For some, words need to be found consciously, rather than being immediately linked with subjectivity. Others described how words could feel out of their control, as though being spoken automatically without a sense of choosing one's words or of having them effectively reflect one's thoughts. One participant described his experience of both thoughts and speaking as follows:

> Like the thoughts still seem to kind of flow, but it's like I don't know where the thoughts are coming from. Also, the experience of thought in my head is different, it's like … it feels very blank. I must be having thoughts because I am able to speak to you.

Actions are often experienced similarly, either occurring automatically ("on autopilot") without a feeling of conscious control, or else requiring more control and effort than usual. The body is felt to be strange and lifeless, like an object one is observing rather than intimately known as one's own animate self. For example,

> My movement is sort of automatic, so it doesn't feel like it's me sort of controlling it. Obviously if I need to stand up or something, I know how to do it … but in my mind, I sort of feel disconnected from it, 'cause I don't really feel like it's happening to me.

Bodily experiences and sensations can also feel detached or somehow anonymous: they are experienced, but without a sense that they are happening to oneself (which has been variously termed "myness," "mineness," or "for-meness"). One participant described how visual perception now "feels like I am not looking out of my eyes, it's kind of like a camera, like camera vision." This can be accompanied by a loss of sensitivity to sensations, which feel duller, less vivid, or reduced in intensity, as described above (in *Loss of resonance*). Rather than feeling as though there is something changed about the world, participants experience this as a lack of responsiveness to these sensations. Sensations, e.g., are no longer imbued with the subjective coloring or affective amplification that typically occurs. Indeed, all emotional responses seem blunted or are not felt at all; even when participants' tone of voice expressed frustration or they described anxiety in the interview, they denied feeling those emotions physically or experiencing them as subjective states. All participants nevertheless described a kind of pervasive mood of anxiety, or irritation, or depression, which seems to infuse all experiences rather than being specifically directed toward specific objects or events.

In addition, time appears to pass outside of or without the individual. It can feel as though time has stopped or disappeared, or like one is suspended in time, even though it seems to be passing in the external world. Alterations in the felt sense of time's passing were highly variable, however. For one participant, time seems to be moving faster around him than before: he is unable to connect his experience of stasis with the fact that he is aging or the world is changing. This can also involve not noticing the passage of time at all (several hours seem to pass as quickly as several minutes). As he noted,

> It definitely feels like I'm kind of suspended in time, for want of a better phrase. But then, it's like kind of reality is just not there, and you're just

> kind of floating … It's just kind of, everything's a bit suspended in the moment.

Another participant noted the unique steadiness of time, which no longer appears to change speed in accordance with his mood or interest level. A third found time less reliable that before: sometimes it went faster, sometimes slower, and time no longer provided a reliable medium in which events unfold with relative consistency.

3.3 *Loss of Self*

The third facet or theme concerns a radical change in how participants experience themselves. They feel completely different from how they felt prior to developing DPDR; and this change is often described as a feeling of death or loss of the old self, while no "new" self seems to appear in its place. Instead, they may feel like there simply is no self that exists; there is an "emptiness," a "nothingness," or a "vacuum," and they may need to learn how to live without a self. For example,

> It feels like my old self is completely gone. Like it's died. That old me before that day is dead. That's how I felt like. So I felt like my self is not here. I felt like I don't exist. It sounds weird, but that's how it feels … I feel like I have like no emotion at all, just a complete emptiness, no feeling of anything.

Experiences may be happening, but it is as though they are happening to someone else or to no one.

One participant stated that despite this loss, some aspects of experience remain familiar, such as a habitual way of saying or doing things, or one's typical sense of humor. However, these behaviors or tendencies seem to ring hollow, with the feeling that should accompany them no longer present. Another participant said that this "self-less" state feels like a more accurate way to experience things, as he believed that the feeling of the self is only a "construction" produced by the brain to lend a sense of coherence to a stream of experiences.

Participants described an accompanying feeling of a loss of continuity of the self over time: participants do not feel that past events had actually happened to them, or they feel they will not exist in the future. One participant noted that even events that had just passed seem disconnected from his current self. Although participants could logically understand and see the continuity of their lives, knowing in some sense that they had participated in their

memories or that they will exist in the future, they do not actually *feel* it. The distant past, in which participants had not yet experienced DPDR, often feels as though the events had happened to someone else. For example,

> I know that person was me but it doesn't feel like it was. It feels like those memories, it's like the memory of a dead person or something. And … the further away, the more time goes on, like the harder it is to relate at all to those memories.

More recent events can be quite difficult to recall: because participants are not fully present to or engaged in behaviors or events that occurred since developing DPDR, recent events have the quality of imagination, a dream, or a fictional narrative, not something immediately lived. One participant described this experience as follows: "I feel very disconnected from memories, especially recent memories, but that's probably because the present seems so unreal that the memories of the unreal things seem unreal as well."

3.4 *Commitment to Reality*

The fourth facet or theme concerns the fact that, despite the strangeness of their experiences, and the difficulty of dealing with them, the participants generally do not question the factual reality of the world around them. They appear to have little difficulty understanding taken-for-granted assumptions about the world or generally knowing how to respond appropriately to everyday situations, although it can feel they are acting on "auto-pilot":

> The world around me seems very fake but in my dream kind of autopilot world I am able to function within it. You know I can do things fairly logically and the common sense is as there as it ever was.

Most participants are committed to getting back to how they experienced reality and themselves prior to developing DPDR.

For some participants, these experiences are associated with feelings of fundamental differentness and inferiority compared to others, further highlighting their fundamental sense that these experiences are *incorrect* or pathological:

> I don't really feel superior to others, if anything sort of I feel inferior, I don't really feel the same as them, and I'm often thinking … do they sort of know I don't really perceive people the same and things like that.

Participants also described feeling particularly vulnerable around others due to this differentness, as though they are not in control of themselves or will be

unable to cope adequately with a given situation, and this might be perceived by others. Among these participants, there tends to be an interpretation of DPDR experiences as being "wrong" and disordered, and a strong belief that their previous experiences are "normal." As one participant stated,

> I know that my experience is altered. I know that it used to all feel normal. So I know that it's wrong now. Intellectually, I know the world is obviously the same … it's just my experience of it has changed.

This can be associated with feelings of anxiety, hopelessness, and despair related to how these experiences have affected them and whether they will improve. Indeed, all participants described spending much time thinking about and researching their experiences to try to learn why these experiences have come about and how they can be changed. One participant who experiences more episodic depersonalization did however describe a kind of attraction or pull to "abandon" himself to his depersonalized state, as the person in this state, he said, sometimes seems to perceive things more accurately or to be more "real" than his everyday self.

All participants noted a change in how they respond to their experiences over time, with some becoming more and more distressed as they have realized the severity and longevity of these changes, while others described a certain acceptance of their experience or even appreciation of some of the insights it could bring them.

4 Discussion

The themes or facets described above can be understood as relating to a common general structure, or Gestalt, of DPDR as experienced by our participants: a felt loss of immediate, familiar, engaged experience in the world. Although subsequent research with larger groups of participants would be necessary to determine whether this Gestalt can be generalized to most or all instances of DPDR, the detailed individual data and its in-depth analysis indicate that this is a experientially valid and useful way to begin conceptualizing DPDR and distinguishing it from other similar disorders. Below, we describe this structure in more depth and detail, along with a brief discussion of relevant philosophical and neurocognitive literature on the experience of selfhood and reality, both disturbed and intact. We follow by highlighting some of the specific domains of experience that are implicated in this Gestalt, some of which have been overlooked in contemporary research on DPDR. Finally, we consider the relevance of our findings for distinguishing superficially similar features in DPDR

and schizophrenia-spectrum disorders, and for how this might lead to a clearer definition and more accurate assessment of these two conditions.

4.1 *General Structure of DPDR*

The findings from this project represent a systematic investigation into the experience of this disorder that was facilitated by using two interviews that exhaustively detail a large range of changes in experiences of self and world. Our results suggest that, for our four participants, the experiences associated with a diagnosis of DPDR point to a unique disruption of the underlying engagement with the world. This involves an acute sense of missing the normal immediacy and familiarity of experience, accompanied by a diminished sense of one's own engaged presence as an experiential subject. This engagement is not, itself, an object of our experience, but serves as the implicit yet immediate background that structures our overall experiential field. In these conditions, the experiencing self feels separate and alienated from subjective experience. This can be described variously: experiences are no longer accessed immediately or transparently through the medium of a grounding experiential self; the self is significantly less involved or implicated in those experiences; experiences are no longer imbued with subjectivity. Such alterations involved not only our participants' sense of self and its continuity over time, but also affected the felt vividness and reality of the world, the passage of time, feelings of enticement and motivation, and sense of connection with others.

The various facets of this structure, as described above, illustrate the seeming necessity of subjective involvement in experience for typical functioning in the world. In addition, our analysis suggests that the everyday experience of what it is to be a self, the sense of being "me," is dependent on a kind of immediate involvement in experience. Our participants still had experiences, but it would be necessary to feel them and react to them in an immediate and unmediated way, to fully live them, in order to experience oneself once again as a unique, coherent, continuous person.

Similarly, participant descriptions suggest that it may be the subjective, personal feeling of the world that imbues it with its qualities of realness and meaning: otherwise, things and other people seem alien, distant, unreal, and irrelevant. The changes in motivation, pleasure, and sensitivity described by participants could be viewed as both cause and consequence of their felt lack of resonance with the world. Relationships with others change similarly, such that they were no longer drawn to (or repelled by) others in the same way. Interestingly, this distance from experience was also implicated in experiences of time or temporality, suggesting that subjective engagement in the world may be necessary for a relatively consistent and predictable experience

of the passage of time; and that, in the absence of this involvement, time may be experienced variously (e.g., slowing down, speeding up, passing without awareness). As one participant stated, "It kind of feels like there is no passage of time anymore … I know it is passing but it feels like it is not. It feels like as if nothing has ever happened to me." Another noted, "I don't feel like I exist, and I don't feel like the world exists, so therefore time can't exist."

Our formulation of the participants' experiences of DPDR is consistent with historical and contemporary conceptualizations of the disorder. Billon (2016) suggests that most early clinicians who studied the disorder (including Dugas [1898], Janet [Janet & Raymond, 1898], and Jaspers [1962]) characterized it as "a partial or a total lack of subjective character" (p. 374), although explanations differed significantly (Sierra, 2009). More recently, philosophers have attempted to formulate the nature of this disturbance of subjectivity with greater specificity. Billon (2015; 2016) has described an attenuated phenomenal or subjective character – a diminished sense of *what it is like* to have experiences. Guillot (2017) offers some refined distinctions, suggesting that subjectivity in DPDR maintains its quality of "for-me-ness" (the sense that I am having an experience) but lacks "me-ness" (the awareness of oneself in having an experience) and "mineness" (the awareness that an experience is my own). Similarly, in the present study, all participants could describe what it was like to have various experiences (related to DPDR or otherwise; intact "for-me-ness") but frequently described feeling as if the experiences somehow did not belong to or come from them (lack of "mineness"), and often described the sense of lacking a self who could have experiences (disturbed "me-ness").[3] Finally, Ciaunica, Charlton, and Farmer (2021) argue that depersonalization reflects an alteration to the *transparency* that typically characterizes conscious experience: awareness of one's acts of consciousness is heightened, and there is an increase in the feeling of being at a greater distance from the contents of consciousness. Both of these experiences were clearly observed in our own participants.

This view of DPDR as involving a disrupted engagement of self in experience is also consistent with recent findings in neuroscience. A review by Sierra and David (2011) found that persons with DPDR demonstrated increased activation of the prefrontal cortex, which is involved in (among other functions) the evaluation of negative information and exerting control over emotional expression, together with reduced activation in the insula and limbic areas, which are responsible for emotional processing. The authors suggested that this combination would mediate a subjective experience of emotional numbing and perceptual detachment, that is, a lack of subjective response to and a distancing from experience. This could suggest that the immediate, vital, and

rich subjective experience of everyday life involves what might be called an affective component or sense of emotional engagement with the things that move us. When affective responses are suppressed, as may be the case for those who feel chronically overwhelmed by their responses to the world, it may have the effect of making everything feel less real and more distant.

The work of philosopher Bruce Bégout (2005) on the *quotidien*, or everyday world, is also of relevance. The key feature of the everyday, Bégout explains, is its taken-for-granted familiarity, which allows us to assume that our worlds are nothing more than they appear. Our socially-determined, everyday habits of perception and interpretation allow us to believe – and to trust the belief – that things are indeed exactly as they seem, exactly as we have always encountered them. This familiarity and regularity allows us to feel a fundamental sense of *trust* that we exist and the world exists.

We would suggest that for our participants, DPDR involves, at the same time, a loss of this immediately felt everyday familiarity *along with* a persistence of this familiarity as a kind of background awareness or quasi-remembrance. Though our participants have lost their familiar, habitual imprinting of the self onto the world, they also "know" that that familiarity *should* be there, indeed perhaps "*is*" still there, if only they could recapture it. The world and the self are, in some fashion, still known to be real, largely because in the past they have always been felt as such – and this fact, grounded in memory, is still somehow registered in the present. Tragically, however, that reality can no longer be accessed because the personally-imbued habits of perception and cognition that bestow it are no longer operating. Speaking about the explicit feeling of familiarity, one participant put it well: "So it's familiar in some sense, if it wasn't familiar in some sense, then you'd just like you'd just leave the house and try to find where your real house is … So there's familiarity in a factual sense, but not in a feeling sense."

4.2 *Symptom Domains of DPDR*

Our results are consistent with previous descriptions of DPDR. In a historical review, Sierra and Berrios (2001) find that symptoms of the disorder fall into five major domains, all of which are represented in our data: changes in body experience, automaton-like feelings (loss of agency), emotional numbing, changes in imagery and memory, and changes in visual perception.

In addition, our participants reported the following experiences that are *not* fully encompassed within these common symptom domains, namely, changes in the quality of thinking, including difficulty accessing thoughts, having fewer, slower, or no thoughts at all, and difficulty connecting thoughts to language. Beyond simple changes in embodiment, the ability to initiate action or speech

was also altered, such that speaking and moving seemed to happen too automatically or required greater control than usual. Participants also described various changes in the passage of time (though this is included among the clinical features of DPDR listed in the DSM5 (APA, 2013)). A strong sense of losing one's identity or feeling discontinuous with one's previous identity was also reported. Furthermore, we found that beyond simple alterations in visual perception (e.g., blurriness, flatness, altered distance or size of objects; APA, 2013), the overall atmosphere or feeling for the world seemed to change, such that the world felt dead, dull, distant or unfamiliar – in ways that could be described, somewhat paradoxically, either as a heightening or as a diminishment of its reality. Participants described how things failed to draw and hold their attention or motivate them to act in certain ways. Furthermore, there was relatively frequent mention of anxiety and insecurity related to one's symptoms, related to fears of being unable to cope with or respond adequately to a given situation. Finally, we would suggest that although it is not in itself a pathological symptom, the persistent awareness of, and a certain lingering commitment to, everyday reality plays an important role, given that an awareness of its attenuation was linked to much of the distress and dysfunction reported by DPDR participants.

A recent study by Ciaunica et al. (2022), conducting open-ended surveys with participants with high levels of trait depersonalization, similarly found a number of areas of altered experience not typically mentioned in the clinical literature. As with the current study, these included alterations of temporality, heightened sensory sensitivity, self-concept, relationship with others, language and communication, attention and cognitive organization, insecurity and social anxiety (though this was less specific to depersonalization symptoms than in the current study), and altered mood (hopelessness, demoralization, and anxiety). Those participants also described various forms and degrees of erosion of self/world boundaries (including feeling as though others are influencing one's actions or thoughts, feeling able to predict others' thoughts and behaviors, feeling as though objects are an extension of one's body, feeling like one's own thoughts or memories are projected into the world with varying degrees of belief, etc.), which were not prominent in our own study. Taken together, these two studies suggest that some features of DPDR may be frequently overlooked in the clinical literature and in clinical practice, and that these should be considered more closely in future research, assessment, and clinical interventions.

Perhaps even more important, though, is the relationship between these symptoms and the participants' experiential core or Gestalt identified in this paper. The clinical Gestalt colored all aspects of the participants' experience:

cognition and perception, affectivity, identity, behavior, and motivation. While certain features were prominent in some individuals compared to others, understanding DPDR as, at its core, the sense of loss of immediate, familiar engagement in experience may give meaning to each symptom, and may ultimately help to more clearly distinguish them from other disorders, as we will see next.

4.3 *Disturbances of Self and World in DPDR and Schizophrenia*

Persons with schizophrenia can also feel disconnected from their own body, thoughts, language, and general sense of self, and they may feel that their experience of self and world have been profoundly changed. They have also scored higher on measures of dissociation and depersonalization than the general population (though lower than persons with DPDR diagnoses; Johnson, 2020). Furthermore, people with DPDR describe their experiences in ways that seem strikingly similar to some of the features thought to be characteristic of schizophrenia. One recent self-report study found self-disturbance scores among participants with high levels of depersonalization that were similar to those found among persons at risk for developing psychosis (though lower than individuals currently experiencing psychotic disorders; Ciaunica et al., 2022; Cicero, Neis, Klaunig, & Trask, 2017). As noted above, a review of first-person narratives and rich clinical descriptions of DPDR (Sass et al., 2013) similarly found many forms of self-disturbance in this literature, though with several notable absences.

Such overlaps might call into question the validity of these disorders as distinct diagnostic categories. The findings in the current study, however, offer a new way to conceptualize these differences. Notably, our participants with DPDR all described a basic "commitment to reality." Even when their perceptions and embodied experiences were significantly altered, they were aware of and maintained a strong belief in the conventions of socially agreed-up reality. Unlike many persons with schizophrenia, the research participants still trusted this vision of the everyday. Indeed, it may well be the commitment to reality as typically conceived, that is, to common-sense and shared or intersubjective reality, and the felt conflict between this commitment and these new and anomalous experiences, that causes so much distress and disruption.

Persons with schizophrenia can and do, of course, also maintain strong ties to conventional reality, may show varying degrees of so-called "reality-testing," and frequently describe a phenomenon known as "double-bookkeeping," in which belief in both everyday reality and a delusional or solipsistic reality are maintained simultaneously (Sass & Pienkos, 2013). The difference here is not that there is no awareness of the conventional or mundane reality in schizophrenia, but rather that there is the acceptance or allowance of one or more

alternative experiential worlds, with different meanings, laws, and truths than those that are shared by other members of society. It is as if, in schizophrenia, everyday shared reality can readily be experienced as but one among two or more optional universes, with no greater claim to shared reality than the alternative worlds. This experience may wax and wane over time in schizophrenia, and at times persons with schizophrenia may accord greater significance and even greater reality to their private, delusional world than they do to the everyday and shared reality that they nevertheless continue to recognize. This tendency does not seem to occur in DPDR: despite the profound alterations in the feel of the everyday that is characteristic of the DPDR individual, and that was described by our participants, the sense of commitment to shared reality is retained.

It may be, then, that this *lack of trust* in common-sense reality or *lack of commitment* to intersubjective reality is a key factor that distinguishes schizophrenia, that may even be unique to it. Any anomalous experiences in this disorder are, so to speak, imprinted or impregnated with this lack of trust or fundamental instability. As Parnas (2011) notes, the clinical core of schizophrenia has classically been characterized as detachment from reality and withdrawal to private life (Eugen Bleuler), loss of vital contact with reality (Minkowski), and crisis of common sense (Blankenburg) – as well as, more recently, disturbance of basic self-experience (i.e. of existing as a coherent subject that is immediately present to itself and its experiences; Sass & Parnas, 2003). Embedded in each of these conceptualizations is the interwoven nature of destabilized self and ungraspable world. Among our participants with DPDR, by contrast, any unusual experiences were accompanied by a fundamental trust in the everyday that persists in the background, along with the jarring awareness of the gulf between one's immediate experience and what one has always believed to be true. As one participant described, "I know this is real, and I know that there's a sense of reality there, and I'm trying to convince myself that it's real, even though it's doesn't feel like it," while another stated,

> If I were born like this … if I never had a memory of anything else … then I wouldn't know how it is like to experience anything differently, so maybe I'd think this is just how the world kind of is, but I know it's not.

It seems that these participants maintain the basic *perceptual faith* – the faith in the reality of the surrounding world – that Merleau-Ponty (1968) describes as underlying our common-sense or natural attitude experience of the world, even though their immediate experience is somewhat inconsistent with this faith. By contrast, persons with schizophrenia seem to find this basic faith to

be tenuous at best (see Morley, 2003, for a more detailed discussion of perceptual faith and its relation to psychotic phenomena).

Furthermore, we might hypothesize that depersonalized experience and schizophrenic experience may be organized around distinctly different core disturbances, despite sharing many features. Whereas our data, along with the clinical and theoretical literature on the disorder, suggests that pure depersonalization may reflect a profound alienation and distancing from one's immediate experience, schizophrenia seems to reflect, rather, a diminished coherence and stability of one's basic self (what R. D. Laing (1990) calls *ontological insecurity*), and a corresponding instability in one's experience of the world. While this typically schizophrenic instability will *also* create the conditions for feeling separated from one's experience – aptly described as an inability "to resonate with the world" or "lack of immersion in the world" (Parnas, 2011, p. 1123) – the separation or non-resonance that occurs in schizophrenia is uniquely bound up with not feeling a coherent sense of self who can have these experiences.

The situation appears to be somewhat different in depersonalization: here too our participants felt their self has been profoundly changed or even disappeared, but this seems primarily to reflect a lack of personal affective coloring or familiarity of experiences, and the way this lack undermines their feelings of continuity and consistency over time. As one participant described,

> I felt like my self is not here. I felt like I don't exist. It sounds weird, but that's how it feels … I feel like I have like no emotion at all, just a complete emptiness, no feeling of anything.… The really core part of depersonalization … [is] the complete emotional numbing. It is like living in complete void of any kind of emotional experience.

These findings are also reflected in the quantitative results, which show few or no responses in the domains of Demarcation/Transitivism or Existential Reorientation on the EASE. Similarly, responses to the EAWE showed that the most prominent alterations were in the realms of Time and Atmosphere, with again few or no responses in Existential Orientation (which overlaps with the EASE domain of Existential Reorientation). This indicates a lack of more profound disturbance of selfhood among our participants, one that calls into question the very boundaries of self and not-self, and the retention of a fundamental commitment to the independent existence of the world and a common-sense understanding of reality.

Similarly, in the research by Sass et al. (2013) noted above, forms of self-disturbance characteristic of schizophrenia that were *not* found in the literature describing DPDR, were those that suggested fundamental cognitive disruption and perceptualization of thought, confusion with the world and

other people (profound disturbances of boundaries), experiences of solipsistic centrality or grandiosity, and profound changes of self-identity (involving confusion about one's gender or age). Here too, the very existence or reality of the self and the world are called into question at a fundamental experiential level in schizophrenia. This form of doubt is also *not* found in DPDR in the current study, nor did we find the severe cognitive disruptions and perceptualization of thought that might be associated with it.

This characterization allows us to revisit the hypotheses posed above about the relationship between depersonalization and schizophrenia/psychosis. Contrary to the first two hypotheses, our data suggests that DPDR, as described by our participants, is likely *not* a schizophrenia spectrum disorder (hypothesis 1), nor are all psychotic symptoms forms of dissociation (hypothesis 2). It seems more likely that DPDR reflects a distinctive core disturbance that can result in different illness trajectories and respond differently to treatments. Some dissociative processes (including depersonalization and derealization) may be involved in the development of some psychotic symptoms, and so hypothesis 3 (increased vulnerability to psychosis) would be worth future exploration. However, if our findings are generalizable to other cases of DPDR (something which requires further study) we would again suggest that individuals with a primary diagnosis of DPDR are unlikely to develop schizophrenia or any type of psychosis, due to the fundamental commitment to reality (and perhaps, the lack of cognitive disorganization that frequently characterizes schizophrenia, which may itself be a consequence of an altered stance toward the world; Sass, 2017) that is a significant element of the DPDR Gestalt. We would then suggest that hypothesis 4 – depersonalization considered as a common symptom that can be found in psychotic disorders – may be a better way to understand the relationship between psychosis and depersonalization as a symptom. Similarly, regarding hypothesis 5 (depersonalization as essentially related to the structure of schizophrenia, though found in other disorders and also a disorder in its own right), it appears that depersonalization in schizophrenia is likely to be a consequence of the fundamental doubt in reality we propose here; this may have an essential, constitutive role in the structure of the disorder, but this cannot be answered by the current paper and will need future research to address.

4.4 *Clinical Implications*

Because of all the domains of experience that are affected, it may be difficult to try to capture all the ways a person might experience depersonalization and derealization. Some features are likely to be more common than others, such as those noted in Sierra and Berrios's (2001) review, described above. But rather than advocating for ever longer lists of symptoms or more items on assessment

instruments, it may be more appropriate for clinicians and researchers to have a clear understanding of what constitutes the DPDR Gestalt – something which we propose here but which requires additional research to determine its validity beyond our limited sample size – and to be aware of its possible manifestations, e.g., in cognition, memory, identity, interpersonal relationships, temporality, etc.

Furthermore, common tools used to assess disturbances in DPDR (such as the Cambridge Depersonalization Scale [CDS]; Sierra & Berrios, 2000) or even phenomenologically sensitive tools like the EASE or the EAWE, may suffer from the failure of many items to capture the phenomenological differences between these alterations as they occur in DPDR and schizophrenia (or, potentially, other disorders). Future work identifying the features that are highly specific to DPDR (e.g., in the CDS) or to schizophrenia (e.g., in the EASE or EAWE), or that refines current item descriptions to be more reflective of these core Gestalts, may be important to avoid potential misdiagnosis.

4.5 *Limitations*

As this study relied on two semi-structured interviews that were designed to reflect experiences reported in schizophrenia, it is possible that some features unique to depersonalization were overlooked during the interviews. However, the questions in both interviews are comprehensive, and their semi-structured nature encourages the interviewer to explore any experiences noted by the interviewee. Furthermore, the interviewer received feedback from participants after initial data analysis, in which they had the opportunity to correct any inaccuracies and add any missing information. Still, it is possible that some experiential features not specifically inquired about might have been missed. Furthermore, as participants were asked to speak about past experiences, their recollection might have been altered due to time elapsed since the original experience and/or to the influence of various interactions or other experiences on their recall. These are potential problems with any study relying on participant reporting of subjective experiences, as has been discussed in Pienkos, Škodlar, and Sass (2021).

An additional consideration is that three of the four interviews were conducted over VOIP. Although this allowed the interviewer to observe the facial expressions and some physical movement of the interviewee, it is possible that important inter-corporeal data was not available through this format. However, it should be noted that the interviews are focused on the *interviewee's* described experience, rather than the interviewer's external assessment and evaluation of the interviewee, so the lack of this information should not significantly limit the data. The wide-ranging semi-structured nature of these interviews, along with the clinical and phenomenological training of the interviewer, permits

perhaps greater sensitivity to the features of the experience-as-lived than other methods (such as self-report measures and unstructured interviews).

Furthermore, diagnostic status was established by participants' self-report of diagnosis by a licensed clinician. Details provided by participants about the process by which they received this diagnosis indicated that all were given by a clinician with appropriate training and expertise to do so. As each was given a diagnosis by a different professional, these diagnostic procedures cannot be said to be standardized. However, the clear descriptions of participants' experiences and symptoms were consistent with a diagnosis of DPDR. It is unlikely that any of the participants had a schizophrenia spectrum disorder, as they had all experienced these particular for at least 15 months and did not describe any psychotic or subpsychotic features.

In addition, only four participants with DPDR were included in this study, all of whom were White males. It is possible that some features were contingent on the gender or racial identity of participants. This may have been partially addressed with the use of imaginative variation in conducting descriptive phenomenological analysis, which is aimed at determining the essences of a particular experience (i.e., of being depersonalized). Clearly, however, a wider range of cultures and life experiences might challenge some of these findings, and this should be addressed in future research.

Commentary on the value of small n and case study methodologies indicate that detailed investigation of a rich data set, such as can occur with small numbers of participants, allows researchers to determine what is and is not possible in a particular phenomenon (e.g. finding one black swan indicates that not all swans are white; Flyvberg, 2006). Although the small number of participants in this particular study does not allow generalization with total confidence to all cases of DPDR, it does indicate the possibility for there to be substantial overlaps between DPDR experience and the altered self- and world-experience in schizophrenia, and also indicates some ways that these may be distinguished. Future research with larger pools of participants can help establish the generalizability of these claims.

5 Conclusions

The in-depth interviews presented and analyzed in this paper point to a single Gestalt of DPDR found among our participants, which we have summarized as a *felt loss of immediate and familiar engagement in experience*. This Gestalt involves a dampening of personalized affective resonance with the world and a distancing from subjective experience. Specific experiences no longer have the familiar affective "flavor" that the subject typically imbues them with. Our

fundamental sense of ourselves as consistent and familiar individuals, the stability and reality of our perceptions, our ability to function with ease in the world, even the basic features of reality such as the passage of time are all profoundly disrupted by this shift. At the same time, there is a clear memory and awareness of what *should be*: participants seem to remember their familiar, everyday, personalized experience, and are painfully aware of its apparent absence in their lives. Many participants stated that living without this subjective resonance hardly feels like living at all: "It feels like I've sort of been robbed of my life the last … two years that I've had this."

Furthermore, our results show a clear distinction between the Gestalt we describe here and that of schizophrenia, reflecting potentially different experiential origins of depersonalization and derealization symptoms in each of these conditions. As past work on schizophrenia points to the role of diminished stability of basic self and the experienced world, depersonalization and derealization are understood as manifestations or sequelae of this instability, rather than reflecting, more directly, the loss of responsiveness and engagement with experience as in DPDR. This is consistent with earlier work reviewing phenomenological changes in DPDR (Sass et al., 2013), indicating the absence of a more profound self disorder despite wide-ranging disturbances in many domains of selfhood. Our results also illustrate the role of disengagement from the world among our participants with DPDR, coupled with a commonsense commitment to reality (that may be disrupted in schizophrenia). Finally, we would suggest that the development of sensitive assessment tools and the search for etiological factors and corresponding interventions will benefit from targeting these core disturbances, rather than focusing on isolated symptoms in each of the disorders.

6 Declaration of Interest Statement

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

7 Data Availability Statement

Due to the nature of this research, participants of this study did not agree for their data to be shared publicly, so supporting data is not available.

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Notes

1 This study was completed at Rutgers University, the University of Hartford, and Clarkson University. Data was collected at Rutgers University and the University of Hartford, and data was analyzed at Clarkson University. The manuscript was written at Rutgers University and Clarkson University.

2 As Parnas (2011) notes, "a Gestalt is a salient unity or organization of phenomenal aspects. This unity emerges from the relations between component features … but cannot be reduced to their simple aggregate. The Gestalt's aspects are interdependent in a mutually constitutive and implicative manner and the whole of Gestalt codetermines the nature and specificity of its particular aspects, while, at the same time, the whole receives from the single aspects its concrete clinical rootedness" (p. 1126).

3 Interestingly, though, Clark (2013) suggests that this quality of experience as belonging "to me," is in fact an illusion, noting that Buddhist and other ancient philosophical traditions distinguish the *Self* from the momentary "thoughts, feelings, and images that sweep across our consciousness, and that … briefly absorb us" (p. 921). Although persons with DPDR surely suffer from no longer identifying with their conscious experience, Clark suggests that the notion of this as pathological may be an artifact of a particularly modern and Western understanding of consciousness and selfhood.

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